PICKING THE RIGHT COUGH SYRUP

It's best to go with the doctor's advice to avoid adverse reactions



MATCH TO SYMPTOMS

Dry cough: suppressant (e.g. dextromethorphan)

Wet cough: expectorant (e.g. guaifenesin)



AVOID DOUBLE DOSING

Check ingredients for paracetamol, antihistamines (diphenhydramine/ chlorpheniramine) or decongestants (phenylephrine/ pseudoephedrine)

Don't mix with similar tablets



BUY SMART

From licensed pharmacies

Check expiry, batch no., and dose by weight, not spoons

Stick to prescribed brand



DROWSY OR NON-DROWSY?

Antihistamines cause sleepiness, blurred focus

Use non-drowsy by day; avoid driving if drowsy



SPECIAL CASES

Under 6 years: use only with doctor's advice

Pregnant/breastfeeding or on antidepressants: check interactions

Diabetic: choose sugar-free, low alcohol

TOP BRANDS

- Benadryl (JNTL)
- Corex-DX (Pfizer)
- Ascoril (Glenmark)
- Chericof (Sun Pharma)
- Grilinctus (Franco-Indian)

On paper, the revised Schedule M—India's 'Good Manufacturing Practices (GMP)' guidelines for pharmaceutical products—sets rigorous new standards encompassing qualified personnel, validated processes and stronger inhouse quality checks. Yet the weakest link remains raw material sourcing. The WHO and industry experts agree that every container of high-risk solvents must undergo identity testing for DEG/EG contamination, with only a 0.1 per cent detection threshold. Plants that rely on supplier paperwork instead of testing risk letting industrial-grade toxins infiltrate the line.

Licensing should be the first gatekeeper. To make a syrup, firms need a state manufacturing licence, proven Schedule M compliance and inhouse or contracted testing arrangements. The framework is sound; enforcement is not. Many small units operate with thin quality assurance. Their real assurance is that state inspectorates are overstretched and most batches will reach shop shelves unmolested. Malini Aisola, co-convenor of the All India Drug Action Network, stresses the need for central oversight. "Anyone can register a company and make a medicine. There must be a centralised monitoring system for all manufacturers, regardless of size or location," she says.

OVER THE COUNTER MISUSE

If contamination kills episodically, misuse harms daily. India's over-the-counter (OTC) culture, crowded labels and the belief that "a syrup is a syrup" fuel self-medication. Codeine restrictions curbed its misuse as an intoxicant, but shifted demand to dextromethorphan, particularly among adolescents. "Cough syrups are widely used in India to relieve cough caused by infections, but misuse and overuse have become a growing concern, particularly among youth," says Dr Rinki Kakkar, senior consultant, internal medicine, Kailash Hospital, Noida. "Overdose can cause hallucinations, motor impairment and respiratory depression. Most coughs are self-limiting and respond to safer home remedies."

Pharmacists, therefore, are a crucial control point. A September 2025 survey of over 500 pharmacists across Mumbai, Pune and Nashik by the generic medicine pharmacy Zeno Health found that Indians rely on chemists—after doctors—for clarity and affordability. "Our pharmacists are trained to go beyond dispensing. They actively educate customers on the correct dosage, duration and possible side-effects as prescribed by the doctor," says Siddharth Gadia, co-founder and CEO, Zeno Health.

India's cough-syrup disasters are not isolated accidents; they are systemic failures. Arushi Jain, director, Akums Drugs and Pharmaceuticals, stresses that rigour must extend throughout production and distribution. "Ensuring strict adherence to GMP guidelines, robust testing standards and quality oversight across the supply chain is essential to prevent such occurrences," she says. Making pre-release assurance routine for the home market—not just at the border—and turning pharmacies into active health checkpoints may prove to be India's strongest safeguard yet. ■

—with Rahul Noronha and Rohit Parihar